BPM&EA Group Project and Test Plan

Second release · 20 October 2026

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Submission: BPM&EA (UFCEP4-0-3)
Project: Hospital Patient Referral, Treatment and Administration System
Team: A/Ruby, B/Estrella, C/Ender, D/Ryan
Releases covered: `submission-2026-09-29` (initial) and `submission-2026-10-20` (revised)
Repository: https://github.com/Estrella6066773/HW_AISD
Operational model of the revised release: `Hospital_All_Processes_V3.bpmn`, process id `Hospital_All_Processes_V3_C8`
This is the BPM&EA project and test record for the revised release. It replaces `submissions/BPMEA/Group_Project_and_Test_Plan_2026-09-29.pdf` as the current plan; that file stays in the pack as the initial-release record and is not rewritten. Every result below is taken from a file in this repository that was opened and read for this document. A criterion with no execution record is marked NOT EXECUTED, not inferred from source.

1. Submission scope and shared artefacts

BPM&EA and AISD use the same case study and one operational implementation. The executable model, Camunda Forms, Java workers and deployment configuration stay in the repository so that the two submissions cannot diverge into separate copies. This document is the BPM&EA-specific record; the AISD acceptance, design-justification and planning documents are in `submissions/AISD/`.

Shared artefact	Repository path	State in the revised release
Operational BPMN	`Week6 Coursework/bpmn/Hospital_All_Processes_V3.bpmn`	Process `Hospital_All_Processes_V3_C8`. The 29 September model `W02_Hospital_All_Processes_Clean_Lines_Camunda8.bpmn` is retained but is no longer the deployed model.
Camunda Forms	`Week6 Coursework/forms/`	12 `.form` files bound to 22 user tasks. `simple_record` is reused by 11 record-keeping tasks.
Worker module	`Week3 Coursework/hospital-pathway/`	Spring Boot 4.0.5, `camunda-spring-boot-starter` 8.9.0, `pom.xml` targets Java 21; 17 Java source files.
Deployment configuration	`Week3 Coursework/hospital-pathway/src/main/resources/application.yaml`; `config/application.example.yaml`	Runnable configuration plus a copyable template.
Acceptance evidence	`Week3 Coursework/hospital-pathway/evidence/`	Master record, per-case logs, end-to-end JSON and the unit-test run. Listed in section 6.5.
Planning sources	`docs/p3-product-backlog.md`; `docs/p4-work-breakdown.md`; `docs/p6-sprint-backlog.md`; `docs/definition-of-done.md`	Product backlog, task cards, Sprint 1 backlog and the Definition of Done.
Readable exports	`submissions/BPMEA/BPMEA_Operational_Process_Readable_2026-10-19.pdf` (16 pages); `submissions/BPMEA/BPMEA_Strategic_Process_Readable_2026-10-19.pdf` (8 pages)	A reader can follow both models without opening Camunda.

The external payment, scheduling and correspondence services are classroom simulations. No real patient data is used: case references are synthetic and labelled as such in the evidence.

2. Identifiable version and its evidence

2.1 The two tagged releases

Release	Identifier	What it identifies
Initial release	Tag `submission-2026-09-29`, commit `f6fec99`	The 29 September state. Model `Hospital_All_Processes_Simple_C8`, eleven forms, and the results recorded in `Group_Project_and_Test_Plan_2026-09-29.pdf`. That document names `fc6293e` as the implementation baseline it reviewed.
Revised release	Tag `submission-2026-10-20`; read its commit SHA from the tag	The current state. Model `Hospital_All_Processes_V3_C8`, twelve forms, and the acceptance run of 19 October recorded in `Camunda_V3_Acceptance_2026-10-19.txt`.

The revised release is named by its tag throughout this document. Its commit SHA is deliberately not written here: the tag is the identifier a reader is asked to resolve, and the release point is the tag rather than any branch head that later moves.

2.2 The operational model of the revised release

The counts below were read from `Week6 Coursework/bpmn/Hospital_All_Processes_V3.bpmn` and are repeated in `Camunda_V3_Acceptance_2026-10-19.txt` and section 4.8 of `docs/requirements-traceability-matrix.md`.

Element	Value in `Hospital_All_Processes_V3_C8`
User tasks	22, all `assignee="demo"`; 0 `candidateGroups`; 0 `candidateUsers`

Element	Value in Hospital_All_Processes_V3_C8
Service tasks	8
Send tasks	2
Exclusive gateways	27
Timers	0 (timerEventDefinition absent; no boundary event)
Message catch events	1 (PaymentReceived, message Message_PaymentResult)
Forms	22 user-task bindings resolving to 12 distinct formId values, served by 12 .form files in Week6 Coursework/forms/

The form binding audit in Java_Clean_Build_Test_2026-10-19.txt reports 22 user tasks, 12 distinct formIds and 12 files, every formId resolves to a file, every file's internal id equals its filename, and no file is orphaned. simple_record is bound to Redirect, RecordDecline, RecordDna, FundingIssue, AdminEnquiry, ClinicalEnquiry, FinanceEnquiry, AuditReports, UrgentFinance, MonitorLetters and GetDocuments.

2.3 Worker module and engine used for the acceptance run

Item	Value
Module	Week3 Coursework/hospital-pathway
Framework and client	Spring Boot 4.0.5; camunda-spring-boot-starter 8.9.0; pom.xml targets Java 21
Toolchain used	Oracle JDK 23.0.1; Maven 3.9.10
Engine and start	Camunda 8.10.0-alpha5 via c8run (bundled JRE 25), started as c8run.exe start --disable-connectors from D:\OneDrive\MyProjects\AISD_Toolbox\tools\c8run-8.10.0-alpha5
Endpoints	REST, Operate and Tasklist on http://localhost:8080 (demo / demo); gRPC on 127.0.0.1:26500
Deployment	Key 2251799813928657; definition key 2251799813928658; version 1

3. Product backlog and sprint backlog

3.1 Product backlog

docs/p3-product-backlog.md is the authoritative list and the authoritative numbering. It carries 26 initial-release product items and four second-release items, each with priority, related requirement, estimate, first and second owner, dependencies, acceptance conditions, evidence and status. Estimates are first-owner hours. The file was narrowed on 16 October to product work only; ceremony and record-keeping work carries no backlog ID and is tracked in the planning records instead.

ID	Item	Priority	Estimate	First / second owner	Status recorded in the backlog
PB-30	Improved operational BPMN and readable export	P0 (second release)	4h	Ruby / Ender	In progress
PB-31	Verification of the revised solution	P0 (second release)	5h	Ryan / Ruby	Done
PB-34	V1-to-V2 model change note	P0 (second release)	2h	Estrella / Ryan	Done
PB-39	Traceability refresh for the revised solution	P0 (second release)	2h	Estrella / Ryan	Done

The 26 initial-release items are unchanged by the second release. One of them, PB-16 (the i* SD/SR models), is recorded as still open for second-owner review.

3.2 Sprint backlog

Sprint 1 was a planning increment only: no model and no code. Its backlog is docs/p6-sprint-backlog.md; the statuses below are the position recorded there on 16 September, not a claim about the revised release.

Task	Pulled from	Work	First / second owner	Estimate	Status at 16 Sep

Task	Pulled from	Work	First / second owner	Estimate	Status at 16 Sep
T01	PB-01	Roles for referral check; secretaries may request missing papers, not accept	Ruby / Estrella	2h	Done
T02	PB-01	Main referral path: receive, check papers, send to consultant	Ruby / Ryan	3h	In progress
T03	PB-02	Four consultant outcomes; reason and decision-maker required	Ruby / Estrella	3h	Not started
T04	PB-02	No first clinic booking after reject	Ryan / Ruby	2h	Not started
T05	PB-03	Booking request only after acceptance	Ruby / Ender	3h	Not started
T06	PB-03	Confirm a slot; handle no suitable slot	Ender / Ryan	5h	Not started
T07	PB-04	Letter as normal; phone attempt if within two weeks	Estrella / Ruby	3h	Not started
T20	Process work	Upload the Product Backlog and Sprint 1 Backlog; all members in the repository	Ruby / Estrella	2h	In progress

The second release was planned as Sprint 3 and Sprint 4 in `Week6 Coursework/plan/sprint3-4-plan_2026-10-16.md`. Sprint 3's goal was to integrate the improved operational BPMN with the reviewed i* views and the strategic model and to write the change down; Sprint 4's goal was to verify the revised solution against a named baseline and carry the traceability grades forward. PB-30 and PB-34 sit in Sprint 3; PB-31 and PB-39 sit in Sprint 4. The same plan separates product rows, which carry a PB number, from coursework process rows, which do not.

4. Planning evidence

Record	Path	What it records
Definition of Done	`docs/definition-of-done.md`	The checklist an item must satisfy, including an openable artefact, a second-owner review and committed evidence. The file states that the checklist is unagreed until all four members sign.
Task breakdown	`docs/p4-work-breakdown.md`	T01–T20 with first and second owner, estimate, dependency, acceptance conditions and evidence. Estimates are first-owner hours and sum to the backlog estimates.
Second-release plan	`Week6 Coursework/plan/sprint3-4-plan_2026-10-16.md`	Sprint 3 and Sprint 4 goals, part A product work, part B process records, risks, and the 20 October deadline.
Standup records	`Week6 Coursework/process/part7-standup-records.md`	One row per member per standup, with completed work, evidence, next action, blocker, support and handover.
Sprint review records	`Week6 Coursework/process/part8-sprint-review-records.md`	SR1–SR4: what was shown, testing evidence, feedback and backlog changes, plus snapshot tags.
Contribution record	`Est/est：PB-17 贡献记录表.md`	E-001 to E-053: per-member evidence with the path that can be opened.

The records are not complete, and the gaps are kept rather than filled. Standup 3 (12 October) has four rows reading "Not recorded"; Standup 4 (16 October) has one member's row filled. Sprint Review 2 has no record. Sprint Review 3 has its date and snapshot tag `sr3-2026-10-14` but its fields are empty; Sprint Review 4 had no date when this document was prepared. These gaps are stated in `part7-standup-records.md` and `part8-sprint-review-records.md` with the reason, and no later work has been dated back into them.

5. Team roles and first-owner contributions

The four owners follow the shared BPMN rather than four isolated models. C's accepted referral reaches B's visit, A's clinical and funding work follows, and D's change route can return to A or Finance. Shared request-payment publishes the payment-result message, and send-booking-confirmation completes the booking communication step.

Member	First-owner contribution	Current technical evidence	Still open or unexercised

Member	First-owner contribution	Current technical evidence	Still open or unexercised
A / Ruby	P5 clinical care, P7 funding, P8 urgent care, P13 reporting; finance exception routes; strategic model and the V3 operational model	`HospitalPathwayWorkers.java` and `RefundWorkers.java` in `Week3 Coursework/hospital-pathway/src/main/java/io/camunda/demo/hospital/`; `docs/strategic-bpmn/PB-12_strategic-process_2026-09-29.bpmn`; `Week6 Coursework/bpmn/Hospital_All_Processes_V3.bpmn`; `ChargeAmountRulesTest` (2 tests) and `RefundWorkersTest` (10 tests) pass in `Java_Test_Results_2026-10-20.txt`	The role boundary fails at AT-03. No acceptance case drives a refund adjudication, an urgent release, or a report. Refund and charge rules are proven at worker level only.
B / Estrella	P3/P4 visit and P6 treatment booking; shared H2 domain database; the twelve forms	`BookingWorkers.java` and the `domain/` entities and repositories in the same module; `Week6 Coursework/forms/`; the form binding audit in `Java_Clean_Build_Test_2026-10-19.txt`	AT-04 drives a first visit to pending only. No case drives a treatment resource from pending to ready, and no case checks duplicate occupancy rows.
C / Ender	P1 registration, P2 referral and redirect, P9 clinic letter	`LetterWorkers.java`; the `dispatch-clinic-letter` and `notify-referrer` jobs; the two dispatch log lines in the G07 record	G07 proves a dispatched letter on one route. The redirected-referrer branch, the escalation thresholds and a retry are NOT EXECUTED.
D / Ryan	P10 enquiries, P11/P12 care change; acceptance planning	`CareChangeWorkers.java`; `domain/AuditEvent.java` and `AuditEventWriter.java`; `CareChangeWorkersTest` (7 tests) passes in `Java_Test_Results_2026-10-20.txt`; `PB-14_acceptance-run-log_2026-10-19.json` and `PB-18_acceptance-run-log_2026-10-19.json`	No acceptance case enters `ClinicalChange`, an enquiry queue or an audit refusal. The worker's `SKIPPED_UNAUTHORISED` and `SENT_FINANCE_REVIEW_REQUIRED` branches are exercised by a unit test only.

Second-owner review is mixed. The strategic model's review was signed on 19 October (section 7 of `docs/strategic-bpmn/PB-12_abstraction-level_2026-09-29.md`). The i* SD/SR second-owner checklist is still unsigned (the PB-16 row in `docs/p3-product-backlog.md`), and the Definition of Done has no group sign-off (`docs/definition-of-done.md`).

6. Test plan, results and evidence

6.1 Verdict rules

The acceptance cases run against the deployed process definition, not against source. **PASS** means every recorded check in that case met its stated rule. **FAIL** means at least one check did not. **NOT EXECUTED** means no run drove that criterion; it is not a pass and not a partial pass. A unit test, a static check or a source file is named as such and is never substituted for an end-to-end acceptance result. Instance keys are Camunda process-instance keys.

All cases submit values through the REST API `/v2`. The drivers are `scripts/pb14_acceptance.py`, `scripts/pb18_acceptance.py` and `scripts/g07_e2e_acceptance.py` in `Week3 Coursework/hospital-pathway/`. Because the values are posted as JSON, the browser-side behaviour of a form is not exercised: required-field blocks, input patterns and conditionally hidden fields are proven to be defined and bound, not proven to reject anything in Tasklist.

In this section, a path beginning `evidence/` means `Week3 Coursework/hospital-pathway/evidence/`.

6.2 Executed acceptance cases

Case and criterion	Preconditions, data and actions	Expected result and pass-fail rule	Actual status and evidence

Case and criterion	Preconditions, data and actions	Expected result and pass-fail rule	Actual status and evidence
AT-01 main referral path (BR-02, BR-03)	V3 at definition 2251799813928658. Case PB14-20261019-214153-MAIN: register a referral, accept it with a reason, then BookVisit attended.	Accept opens BookVisit and ClinicalCare can follow; the instance does not end rejected; zero incidents. Fail if the route reverses.	**PASS.** Instance 2251799813930310. E1–E5 met: BookVisit after accept, decision accept with its reason, ClinicalCare after slot, not EndRejected, zero incidents. evidence/PB-14_acceptance-run-log_2026-10-19.json.
AT-02 rejected referral (BR-02, BR-03)	Case PB14-20261019-214153-REJECT: register a referral, then reject it with a reason.	Instance completes; decision and reason are stored; no BookVisit; zero incidents. Fail if a booking appears.	**PASS.** Instance 2251799813930247. State COMPLETED, decision reject with reason "Referral not appropriate for this specialty", BookVisit_created false, zero incidents. evidence/PB-14_acceptance-run-log_2026-10-19.json.
AT-03 role boundary (BR-37, BR-09, BR-21, BR-27, BR-31)	Case PB18-20261019-214159-UNAUTH: register a referral, then complete ReviewReferral as accept while staffRole is "Medical Secretary / not authorised to accept".	The engine refuses the unauthorised acceptance and no BookVisit is created. Fail if a booking appears.	**FAIL.** Instance 2251799813930421. U1_BookVisit_created_after_secretary_accept is true and U1_engine_refused_unauthorised_accept is false. Cause recorded in the log: "All user tasks assignee=demo; no Camunda candidate-group RBAC in classroom model". evidence/PB-18_acceptance-run-log_2026-10-19.json.
AT-04 no suitable slot (BR-10, BR-11, BR-35)	Case PB18-20261019-214159-PENDING: register, accept, then BookVisit with outcome pending.	A pending visit is not confirmed and does not reach ClinicalCare; outcome recorded as pending; zero incidents. Fail on early confirmation.	**PASS for the pending slice.** Instance 2251799813930493. The BookVisit task appeared again after the pending outcome, ClinicalCare was absent, visitOutcome was pending, zero incidents. The treatment-resource half (pending, then ready, then one CONFIRMED row) was NOT EXECUTED. evidence/PB-18_acceptance-run-log_2026-10-19.json.
AT-05 failed payment and deliberate retry (BR-14, BR-17)	Case PB18-20261019-214159-PAYFAIL: funding at 10.00 fails to investigate, FundingIssue recorded, retry at 10.01.	After failure payment_status is investigate and BookTreatment is not reached; the retry succeeds and BookTreatment is reached. Fail on a second automatic charge.	**PASS.** Instance 2251799813930607. payment_status investigate after the failure, successful after the retry, BookTreatment reached. The payment is mocked: no real provider charge and no database row count are claimed. evidence/PB-18_acceptance-run-log_2026-10-19.json.
AT-06 payment taken without confirmation (BR-18)	Case PB18-20261019-214159-NOCONF: funding with the investigation outcome.	payment_status is investigate, a human Funding issue opens, no automatic BookTreatment and no second request. Fail if the process books automatically.	**PASS.** Instance 2251799813930876. payment_status investigate, payment_requested_once false, BookTreatment not automatic. evidence/PB-18_acceptance-run-log_2026-10-19.json.

Case and criterion	Preconditions, data and actions	Expected result and pass-fail rule	Actual status and evidence
G07 full self-pay route (BR-02, BR-03, BR-14, BR-16, BR-30)	V3 and twelve forms deployed, workers running. Case `GROUP-E2E-20261019-214234`: accept, attend, approve treatment, dispatch the letter, self-pay 10.01, reserve the treatment slot, discharge, dispatch the final letter.	Correlated payment result, all required workers execute, no incident, nine human tasks completed and the instance COMPLETED. Fail if it stalls or ends wrongly.	**PASS.** Instance `2251799813931091`, COMPLETED, nine human tasks, `activeIncidents` empty. Final variables: `payment_status` `successful`, `booking_slot_status` `CONFIRMED`, `fundingStatus` `patient_paid`, `clinicLetterDispatchStatus` `dispatched`, `confirmation_sent` true. All ten assertions true. `evidence/Group_Final_E2E_20261019-214234.json`.
Letter dispatch on the G07 route (BR-30, BR-31, BR-32)	The G07 route completes DispatchLetter twice with `letterStatus` sent.	One mock dispatch receipt per action and stable references; no real delivery claim. Fail if a dispatch is missing.	**PASS on the G07 slice.** Two `dispatch-clinic-letter` lines in the master record, at 21:42:38.594 and 21:42:41.358, both `status=sent` `ref=C-DISPATCH-GROUP-E2E-20261019-214234`; final `clinicLetterDispatchStatus` `dispatched`. Redirected referrer, escalation and retry were NOT EXECUTED. `evidence/Camunda_V3_Acceptance_2026-10-19.txt`.

The master record `Camunda_V3_Acceptance_2026-10-19.txt` gives the deployment key, definition key and each instance key, and prints the raw driver output for all six cases and for G07. The two per-case JSON logs are the primary evidence for AT-01 to AT-06; the end-to-end JSON is the primary evidence for G07.

6.3 Environment event during the run

The first c8run start, without `--disable-connectors`, was shut down at 21:38 because the bundled Connectors runtime never became healthy. One G07 attempt died at 21:38:33 with an engine connection reset; it is retained as `Week3 Coursework/hospital-pathway/evidence/Group_Final_E2E_20261019-213833.json`, instance `2251799813929726`, with no completed tasks and result FAIL. The engine itself stayed healthy. Every case was re-run in one continuous session from 21:40 with `c8run.exe start --disable-connectors`, and all results in section 6.2 come from that session. A reader reproducing the work needs the flag: without it the connector health check can terminate the engine mid-run, which looks like a process failure but is not one.

A second retained file, `Group_Final_E2E_20261019-214113.json`, records instance `2251799813929943` as COMPLETED with the same variables but an empty task trail. It is kept but is not cited as a result; the cited G07 run is `2251799813931091`.

6.4 Criteria not exercised

These criteria were never driven by an acceptance case. Each row states what a case would have had to do and what remains proven only by source or by a worker unit test.

Criterion	What a case would need to do	Status
Authorised clinical change, `SKIPPED_UNAUTHORISED`, `SENT_FINANCE_REVIEW_REQUIRED` (BR-19, BR-22)	Enter `ClinicalChange`, submit an unauthorised change, and route a paid change to Finance.	**NOT EXECUTED.** The form and worker exist; the two outcomes appear only in `CareChangeWorkersTest` log lines, which is unit evidence.
DNA and patient cancellation (BR-36)	Drive `RecordDna` or `RecordDecline`; for a paid case reach `FinanceAdjustment`.	**NOT EXECUTED.** The tasks and form outcomes exist; no case enters them.
Urgent authorisation and urgent finance (BR-19)	Enter `UrgentAuthorisation`, record a clinical reason, then create the Finance follow-up.	**NOT EXECUTED.** The form and tasks exist; AT-05 drives the ordinary retry, not the urgent bypass.

Criterion	What a case would need to do	Status
Refund adjudication (BR-20)	Reach `FinanceAdjustment`, select one of the five decisions, and reject an invalid partial amount.	**NOT EXECUTED.** `finance_adjustment.form` and `RefundWorkers.requirePartialAmount` exist; `RefundWorkersTest` covers them at worker level only.
Letter escalation (BR-32, BR-33, BR-34)	Hold a letter past 7 days, then 1 month and 3 months, and observe escalation and reminder stop.	**NOT EXECUTED.** `MonitorLetters` is a human task and the model has no timer, so the threshold cannot be reached by waiting.
Review booking and follow-up (BR-35)	Request a follow-up period and drive a no-slot outcome to the pathway team.	**NOT EXECUTED.** No case requests a follow-up appointment.
Pending-to-ready treatment resource (BR-10, BR-11)	Drive a treatment resource to pending, then ready, and inspect occupancy.	**NOT EXECUTED.** AT-04 covers a first visit in `pending`; it does not drive `BookTreatment` to pending and back.

Two further limits apply to every row above. **Browser-side form validation is NOT EXECUTED**: the drivers post values over REST, so a constraint such as `disallowPassedDates` in `book_visit.form` was never triggered, and form constraints are proven to be defined and bound rather than proven to reject anything in the UI. **Timer-dependent criteria cannot pass**: the model contains no `timerEventDefinition`, no boundary event and no event-based gateway, so any rule that depends on elapsed time has no mechanism to satisfy it.

6.5 Test evidence files

- `evidence/Camunda_V3_Acceptance_2026-10-19.txt` — master record: engine, deployment, instance keys, final variables, assertions, worker-log excerpt, raw driver output and the environment event.
- `evidence/PB-14_acceptance-run-log_2026-10-19.json` — AT-01 and AT-02 tasks, checks and verdicts.
- `evidence/PB-18_acceptance-run-log_2026-10-19.json` — AT-03 to AT-06 tasks, checks and verdicts; records AT-03 as the only failed test.
- `evidence/Group_Final_E2E_20261019-214234.json` — G07 instance, tasks, final variables and assertions; result PASS.
- `evidence/Group_Final_E2E_20261019-213833.json` — the failed G07 attempt killed by the connection reset; no tasks, result FAIL.
- `evidence/Group_Final_E2E_20261019-214113.json` — a COMPLETED instance with an empty task trail. Retained, not cited.
- `evidence/Java_Test_Results_2026-10-20.txt` — `mvn -B clean test` at the released tree: BUILD SUCCESS, 19 tests, 0 failures, 0 errors, 0 skipped; `CareChangeWorkersTest` 7, `ChargeAmountRulesTest` 2, `RefundWorkersTest` 10. JDK 23.0.1, Maven 3.9.10; `src/main`, `src/test` and `pom.xml` unchanged since `dbc0a33`.
- `evidence/Java_Clean_Build_Test_2026-10-19.txt` — the earlier clean build and the form binding audit: 22 user tasks, 12 distinct formIds, 12 files, all resolve, no orphan.

6.6 Coverage profile

`docs/requirements-traceability-matrix.md` grades 42 business rules. The initial release stands at **12 Supported, 26 Partial, 4 Unsupported**. The revised release stands at **13 Supported, 25 Partial, 4 Unsupported**. One grade moved, BR-20 from Partial to Supported, on the strength of the revised refund form and worker; the V3 acceptance run moved no grade, because no case drove a branch that one of the Partial notes names as its gap.

The four Unsupported rules are BR-37 (role-based access control), BR-39 (patient identity matching), BR-40 (outage back-entry) and BR-42 (communication preferences). BR-37 is the one this report can demonstrate: AT-03 fails on both releases for the same reason, no candidate groups exist, and every user task is assigned to `demo`. It remains Unsupported.

7. Evaluation

The revised release supports one complete self-pay route from referral to discharge on the deployed V3 model, plus five of the six acceptance cases. G07 completed with nine human tasks, no active incident and the payment, booking, funding, letter and confirmation variables all in their expected final state. AT-01, AT-02, AT-04, AT-05 and AT-06 pass, and the letter dispatch step is observed twice on the G07 route. The worker module builds cleanly and its 19 unit tests pass at the released tree.

The release does not support the role boundary. AT-03 is a failure, not a limitation to be read past: a Medical Secretary acceptance still created BookVisit, because the model has 22 tasks assigned to `demo` and zero candidate groups. The same failure occurred in the initial release, so the second release has reproduced it rather than introduced it. BR-37 stays Unsupported, and the rules that depend on role enforcement (BR-09, BR-21, BR-27, BR-31) stay Partial for that reason.

The release does not support any branch that no case drove. The authorised clinical change and its skip and Finance outcomes, DNA and cancellation, urgent authorisation and urgent finance, refund adjudication, letter escalation, review booking and the pending-to-ready treatment resource are all modelled or implemented to some degree and all NOT EXECUTED as acceptance cases. The 13/25/4 profile is therefore unchanged from the revised traceability refresh, and it should not be read as coverage of those branches. Browser-side form validation and every timer-dependent rule are outside what these runs can prove: the drivers post JSON over REST, and the model has no timer.

What remains is specific work rather than a restatement of intent. Candidate groups or an equivalent role check are needed before AT-03 can be re-run with a different expectation. The unexercised branches need acceptance cases of their own, starting with the clinical change and the treatment pending-to-ready transition, because those are the two where the model already contains outcomes the acceptance record has never observed. A Tasklist observation pass is needed before any claim about form rejection is made. Until then this document records six executed acceptance cases, one end-to-end route, one failure and a list of criteria that were not exercised.